Selorah Health

The operating system for health records in Africa

One card. One identity. Any hospital. With or without internet.

Samuel Ikechukwu Amanze ▪ Founder ▪ 2026

THE PROBLEM

Every new hospital starts from zero

Records don’t travel

Hospitals cannot (or will not) share a patient’s history with another facility, even when the patient requests it.

Repeat everything

New card. New basic tests. New needles. Lost papers. Delays. Cost. Risk, especially in emergencies.

Women pay a higher price

Pregnant patients or those managing hormonal issues cannot easily change location without re-starting the clinical baseline.

WHY SELORAH EXISTS

Two personal moments

01

The accident

A friend was rushed from a car accident. The receiving hospital required a new card and basic tests before treatment. She did not survive. Critical minutes were lost to process instead of care.

02

The pregnancy

A pregnant colleague needed to relocate. She stayed in an unsuitable place rather than risk another full set of baseline tests and needle pricks at a new hospital. The system forced her to choose location over continuity.

THE SOLUTION

A card that works with or without internet

1 Physical card

Front: unique QR. Back: emergency info (allergies, blood group, contacts, key vitals).

2 Good internet

Scan QR → pull latest relevant records & emergency profile for the clinician.

3 No / poor internet

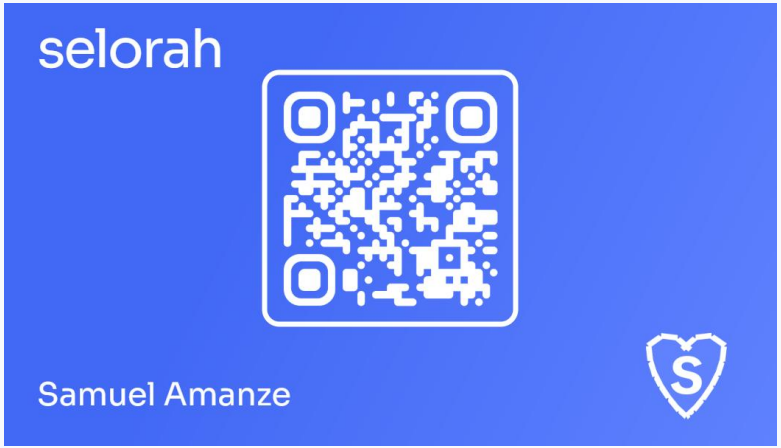
Use the printed back of the card immediately. No app required for the emergency layer.

4 Hospital EMR

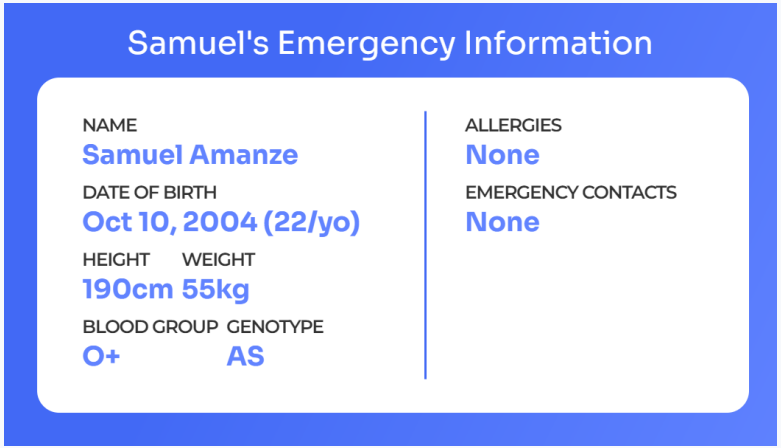
Search any patient by NIN inside a unified system. Role-based portals for staff.

THE CARD

Designed for the first 60 seconds of care



FRONT — QR code + patient name



BACK — Emergency medical information

Prototype cards printed. Production-quality version is the next hardware step.

WHO BENEFITS

Four sides of the same table

Patients

Own their data. Share securely. Use any hospital without starting over. Earn from consented research.

Hospitals

Unified EMR search by NIN. Faster emergency handovers. Share of research revenue.

Researchers

Legitimate, consented, anonymised data at scale; not grey-market fragments.

Insurers

Cleaner claims signals and better risk insight when records are on shared rails.

TRACTION

Past ideation. Prototype live. Early signal strong.

39	4.1	82%
Survey responses (waitlist signal)	Avg interest score (out of 5)	Open to blockchain-secured records*

* “Yes, definitely” + “Yes, probably” from survey. Top stated frictions: hospital delays, lost papers, privacy.

Live demo & waitlist: selorah.vercel.app

Physical card designs complete. Prototype cards printed. Early clinical input from a practising pharmacist on emergency data needs.

HOW EVERYONE EARNS

Aligned incentives, not extraction

When researchers / pharma pay for consented anonymised data

50%

Selorah

Platform & compliance

30%

Patients

Who contributed data

20%

Hospitals

Whose patients participated

Also: Hospital SaaS ▪ Patient subscription (includes card) ▪ Gamified onboarding rewards that build data density

Early payers: hospitals + research organisations. Patient growth is the flywheel.

WHERE WE ARE

The bottleneck is professional execution

- ✓ Problem validated through personal experience + survey
- ✓ Working prototype & public demo live
- ✓ Card design complete; prototype printed
- ✓ Waitlist & early interest signal (39 responses)
- Production-grade engineering team
- Hospital pilot partners
- Regulatory & security hardening for scale

We are raising $40,000 in Pre-seed Funding

USE OF FUNDS:

- 50% — MVP Development & MONAD Integration
- 20% — Hospital Pilots & User Testing in Lagos
- 15% — Marketing & User Growth
- 10% — Security Audit & Legal/Compliance
- 5% — Operations & Tools

MILESTONES WITH FUNDING:

- Month 1–3:
  Complete MVP and Onboard first 1500 users + Two Hospital Pilots
- Month 4–6:
  Launch + First Research Earnings Distribution
- Month 6–12:
  Scale to 5,000+ **active** users

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Health records should travel with the patient.

Emergency information should be readable in the first 60 seconds; whether the internet is up or down.

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Website